

Rural Health Transformation - Post- Award Budget Narrative

Wyoming RHT Budget Summary per Initiative FY 2026		
Initiative	FY 2026	Total Funding Per Initiative (%)
Access to Emergency Medical Care	\$110,866,565	48.4%
Rural Workforce Supply	\$56,630,670	25.6%
Health Technology Transformation	\$12,856,333	8.9%
Make Wyoming Healthy Again	\$22,783,382	16.1%
State Administrative Expenditures	\$1,867,792	1.0%
Total	\$205,004,742	100.0%

Wyoming RHT Budget Summary Direct and Indirect Costs, and Cost Type			
Direct/ Indirect	Cost Type	FY 2026	Total Funding Per Cost Category (%)
Direct	A. (Personnel) Salaries and Wages	\$1,311,304	0.70%
Direct	B. Fringe Benefits	\$283,791	0.15%
Direct	C. Travel	\$106,997	0.05%
Direct	D. Equipment	\$0	0.00%
Direct	E. Supplies	\$70,400	0.03%
Direct	F. Consultant/ Subrecipient/ Contractual Costs	\$203,191,950	99.04%
Direct	H. Other	\$40,300	0.02%
Direct	I. Total Direct Costs	\$205,004,742	100.00%
Indirect	J. Indirect Cost Allocation	\$0	0.00%
Total		\$205,004,742	100.0%

Wyoming RHT - A. (Personnel) Salaries and Wages [†]				
Initiative	Position Title	Duties	Wage/hr	FY 2026
Overall Oversight	Pay Add On to Initiative Project Champions	Additional oversight and workload for experienced employees assigned to oversee initiatives, with temporary pay-add on during RHTP. Pay Add-On for 13 middle managers-11 initiatives champions+2 admin/fiscal champions. Only add-in pay portion charged to RHT.	\$5.77 add-on per hour	\$156,000
Initiative Subtotal				\$156,000
Access to Emergency Medical Care	1. CAH Health Program Manager II	CAH Program Design, Procurement, Contracting, Contract Enforcement, Financial Management	\$42.69	\$88,789
	2. CAH Program Manager	CAH Program Design, Procurement, Contracting, Contract Enforcement, Financial Management	\$35.60	\$74,057
	3. EMS Health Program Manager	EMS Program Design, Procurement, Contracting, Contract Enforcement, Financial Management	\$35.60	\$74,057
	4. EMS Senior Grants & Contract Administration Specialist	EMS Procurement, Contracting, Contract Enforcement	\$32.52	\$67,635
	5. EMS Senior Grants & Contract Administration Specialist	EMS Procurement, Contracting, Contract Enforcement	\$32.52	\$67,635
Initiative Subtotal				\$372,173
[†] We anticipate hiring all positions by 7/1/2026				

Wyoming RHT - A. (Personnel) Salaries and Wages [†]				
Initiative	Position Title	Duties	Wage/hr	FY 2026
Rural Workforce Supply	6. Health Program Manager	Health Workforce Program Design, Procurement, Recruitment, Contracting, Marketing, Financial management	\$35.60	\$74,057
	7. Senior Grants & Contract Administration Specialist	Health Workforce Procurement, Contracting, Disbursements	\$32.52	\$67,635
	8. Senior Grants & Contract Administration Specialist	Health Workforce Procurement, Contracting, Disbursements	\$32.52	\$67,635
	9. Senior Grants & Contract Administration Specialist	Health Workforce Procurement, Contracting, Disbursements	\$32.52	\$67,635
	10. Senior Grants & Contract Administration Specialist	Physician Post-Graduate Education Program Procurement, Contracting, Disbursements	\$32.52	\$67,635
	11. Senior Grants & Contract Administration Specialist	Workforce education startup Procurement, Contracting, Disbursements	\$32.52	\$67,635
	Initiative Subtotal			\$412,232

[†] We anticipate hiring all positions by 7/1/2026

Wyoming RHT - A. (Personnel) Salaries and Wages [†]				
Initiative	Position Title	Duties	Wage/hr	FY 2026
Health Technology Transform.	12. Technology Adoption Challenge Grants & Contract Admin Manager I	Technology Adoption Challenge program design, procurement, recruitment, contracting, marketing, financial mgmt.	\$35.60	\$74,056
	13. Statewide Telespecialist Network Senior Grants & Contract Admin Specialist	Statewide telespecialist network procurement, contracting, vendor mgmt, deliverable review, contractor compliance.	\$32.52	\$67,635
	14. Senior Business Project Management Analyst	Transportation coord., routing, and billing, tech system project mgmt, procurement, contract mgmt, IT oversight.	\$35.60	\$74,056
	15. Centralized Billing Capability Project Mgmt Supervisor (PMP certification)	Centralized billing capability IT project mgmt, billing, contract mgmt, deliverable review, contractor compliance	\$38.99	\$81,095
Initiative Subtotal				\$296,843
Make Wyoming Healthy Again	16. Integrated Primary Care Grants & Contract Administration Manager I	Integrated Primary Care Program Design, Procurement, Contracting, Contract Enforcement, Financial Management	\$35.60	\$74,056
Initiative Subtotal				\$74,056
Wyoming RHT	Total Expenditures	16 Positions		\$1,311,304

All positions are new employees of the State of Wyoming, WDH, and are hourly with temporary status and limited benefits budgeted at 2080 hour per year (12 month year), at a 100% level of effort allocation to RHT. Wages set at 90% of market using Wyoming state payscale. All wages budgeted to grow at 5% annually based on recent wage trends. <https://ai.wyo.gov/divisions/human-resources/consultative-services/compensation/pay-tables-salary-averages>

[†] We anticipate hiring all positions by 7/1/2026

Wyoming RHT - B. Fringe Benefits			
Initiative	Position Title	FY 2026	FY 2030
Overall Oversight	Pay Add-On to Initiative Project Champions (13 individuals)	\$10,920	\$10,920
Access to Emergency Medical Care	1. CAH Health Program Manager II	\$18,215	\$22,140
	2. CAH Health Program Manager	\$17,184	\$20,887
	3. EMS Health Program Manager	\$17,184	\$20,887
	4. EMS Senior Grants & Contract Administration Specialist	\$16,734	\$20,341
	5. EMS Senior Grants & Contract Administration Specialist	\$16,734	\$20,341
Rural Workforce Supply	6. Health Program Manager	\$17,184	\$20,887
	7. Senior Grants & Contract Administration Specialist	\$16,734	\$20,341
	8. Senior Grants & Contract Administration Specialist	\$16,734	\$20,341
	9. Senior Grants & Contract Administration Specialist	\$16,734	\$20,341
	10. Senior Grants & Contract Administration Specialist	\$16,734	\$20,341
	11. Senior Grants & Contract Administration Specialist	\$16,734	\$20,341
Health Technology Transformation	12. Technology Adoption Challenge Grants & Contract Administration Manager I	\$17,184	\$20,887
	13. Statewide Telespecialist Network Senior Grants & Contract Administration Specialist	\$16,734	\$20,341
	14. Senior Business Project Management Analyst	\$17,184	\$20,887
	15. Centralized Billing Capability Project Management Supervisor	\$17,677	\$21,486
Make Wyoming Healthy Again	16. Integrated Primary Care Grants & Contract Administration Manager I	\$17,184	\$20,887
Wyoming RHT	Total Expenditures	\$283,791	\$342,596
All positions have the same benefits applied. Fringe Benefit 7%; Composed of 6% FICA and Medicare, plus 1% state benefits including workers compensation. Health Insurance-single individual, \$12,000 annual cost, expected to grow at 5% annually.			

Wyoming RHT - C. Travel							
Travel Type				FY 2026			
Out of State				\$55,664			
In State				\$51,333			
Total Travel				\$106,997			
Travel estimates reflect 15 out-of-state events to national RHT related conferences at approximately \$2,000 per event and 45 in-state events for contractor and sub-recipient monitoring at approximately \$733 per event. Detailed calculations are shown in tables below.							
Expected Travel Events							
Type of Travel	Item	# of Individuals	Trips Per Year	Cost Per Item	Basis	Items Per Trip	Travel Cost
Out of State (budgeted as Washington, DC)	Hotel/ Lodging	28	1	\$275.00	Night	3	\$23,100
	Per Diem (M&IE)	28	1	\$92.00	Day	2	\$5,152
	Per Diems (M&IE)- Travel Days	28	1	\$69.00	Day	2	\$3,864
	Flight	28	1	\$545.00	Round Trip	1	\$15,260
	Local Travel (POV Mileage)	28	1	\$0.67	Mile	100	\$1,876
	Parking & Tolls	28	1	\$19.00	Day	4	\$2,128
	Baggage Fees	28	1	\$51.00	Round Trip	1	\$1,428
	Taxi	28	1	\$51.00	Single Way ride	2	\$2,856
	Total Out Of State						
In State	Hotel/ Lodging	28	2.5	\$110.00	Night	2	\$15,400
	Per Diem (M&IE)	28	2.5	\$68.00	Day	1	\$4,760
	Per Diems (M&IE Only)- Travel Days	28	2.5	\$51.00	Day	2	\$7,140
	Local Travel (POV Mileage)	28	2.5	\$0.67	Mile	512.5	\$24,033
	Total In State						

[FY 2026 per diem rates for Wyoming | GSA](#); [FY 2026 per diem rates for District of Columbia | GSA](#)

Wyoming RHT - D. Equipment

Not applicable to Wyoming's RHT proposal. Contractors or sub recipients may have equipment as part of their proposals, and we note that in F. Contractor Breakdown if expected.

Wyoming RHT - E. Supplies

Supplies Type			FY 2026		
Laptops			\$24,000		
Monitors, Hub and Computer Peripherals			\$12,800		
Training Supplies			\$14,500		
Educational Pamphlets			\$8,300		
General Office Supplies			\$9,600		
Printer, scanner, combo			\$1,200		
Total Supplies			\$70,400		
Supplies	Number of Items	Cost/ Rate Per Item	Basis	Lifespan (Yrs)	Item Cost
Laptop	16	\$1,500	Per Item	5 yrs	\$24,000
Monitors, Hub and Computer Peripherals	32	\$400	Per Item	5 yrs	\$12,800
Training Supplies	725	\$20	Per attendee	1 yr	\$14,500
Educational Pamphlets (Year 1)	4,150	\$2	Per Item	1 yr	\$8,300
General Office Supplies (12 months x \$50/mo x 16 staff)	16	\$50	Per month	12 months per year	\$9,600
Printer & scanner combo	2	\$100	Lease per month	12 months per year	\$1,200

Wyoming RHT- Category F Contractor Breakdown			
Wyoming Budget Summary per Initiative Component (Part 1 of 2)			
Initiative	Initiative Component	FY 2026	Total Percent Funding Per Component
Access to Emergency Medical Care	1.1 Critical Access Hospital - Basic	\$66,749,544	29.14%
	1.2 EMS regionalization	\$44,117,021	19.26%
	1.3 Public major medical plan	\$0	0.00%
Rural Workforce Supply	2.1 Workforce education individual support	\$35,424,820	15.47%
	2.2 Physician post-graduate education	\$17,712,410	7.73%
	2.3 Workforce education startup grants	\$3,193,440	2.36%
	2.4 RHT Policy Initiative- Licensure Compacts Physician Assistants	\$100,000	0.02%
	2.5 RHT Policy Initiative- Scope of Practice (Pharmacists)	\$100,000	0.02%
	2.6 RHT Policy Initiative- Scope of Practice (Dental Hygienist)	\$100,000	0.02%

Wyoming RHT- Category F Contractor Breakdown			
Wyoming Budget Summary per Initiative Component (Part 2 of 2)			
Initiative	Initiative Component	FY 2026	Percent Per Component
Health Tech. Transform.	3.1 Technology adoption challenge	\$6,458,917	4.47%
	3.2 Statewide tele-specialist platform	\$3,493,440	2.42%
	3.2 Independent IT Security Architect	\$10,000	0.00%
	3.3 NEMT coordination	\$828,808	0.57%
	3.3 Independent IT Security Architect	\$10,000	0.00%
	3.4 Centralized billing capability	\$2,075,168	1.44%
	3.4 Independent IT Security Architect	\$10,000	0.00%
Make Wyoming Healthy Again	4.1 Integrated primary care expansion	\$15,613,217	10.81%
	4.2 Clinically-integrated care coordination	\$2,989,078	2.38%
	4.3 Exercise and healthy diet promotion; RHT policies	\$4,181,087	2.90%
Total Expenditures		\$203,166,950	99.03%

Wyoming RHT- Category F Contractor Breakdown			
Wyoming Budget Detail per Initiative, Sub-Component, and Cost Category			
INITIATIVE 1: Access to Emergency Medical Care			
Initiative Component	Cost Type, Cost Category	FY 2026	% of Total RHT
1.1 Critical Access Hospital - Basic	Direct, Subrecipient	\$66,749,544	29.14%
1.2 EMS regionalization	Direct, Subrecipient	\$44,117,021	19.26%
Initiative 1: Access to Emergency Care Summary	Direct, Subrecipient/Contractual Costs	\$110,866,565	48.40%

Wyoming Initiative Component Descriptions and Pertinent Information				
INITIATIVE 1: Access to Emergency Medical Care				
Initiative Component	Description of initiative component	Procurement Expected	Allowable Bidder Pool	Expected Use of Funds
1.1 Critical Access Hospital - Basic	Incentive payments for CAHs to focus on adoption of new more limited CAHB model, as described in the narrative. Payments may be tiered for CAHs meeting various components (maternity, regional ambulance/EMS, etc). Costs for CAH swing bed conversions may include for example: equipment, supplies, and light renovations within existing footprints.	Competitive Request for Application (RFA)	Wyoming CAHs or potential CAHs	Will vary based on RFA responses within allowable categories. No indirect costs allowed.
1.2 EMS regionalization	Incentive payments for CAH, hospital districts, and EMS agencies for regionalization of EMS. EMS regionalization startup payments may include equipment and supplies such as new ambulances, refresh of ambulance area equipment, communications integration with new hub, and refresh of medical equipment and other station area/bays. Incentive payments may pay for regional model adoption, incl. serving regions/areas without sufficient volume to sustain on insurance billing alone.	Competitive RFA	Wyoming CAHs, hospitals, EMS agencies, counties	Will vary based on RFA responses within allowable categories. No indirect costs allowed.

Wyoming Initiative Component: Start-up Funding		
INITIATIVE 1.1: Critical Access Hospital - Basic		
Startup cost (prioritized)	Use of funds	Budgeted amount (of total \$5 million pool)
Restoration of labor and delivery services	Appropriate care availability and workforce	Up to \$750,000 per provider, one time (est. 2 CAHs)
Time-sensitive trauma and stroke imaging equipment purchases and upgrades	Appropriate care availability if not extant or insufficient. May integrate with provider technology adoption challenge	Up to \$500,000 per provider (est. 5 CAHs)
Restoration or start-up of dialysis services	Appropriate care availability, if not extant or insufficient. Integrates with tele-specialist initiative.	Up to \$250,000 per provider (est. 4 CAHs)

Wyoming Initiative Component: Eligible Applicants	
INITIATIVE 1.1: Critical Access Hospital - Basic	
Facility Type	Facility Names
Critical Access Hospital (18 Total)	Carbon County (Rawlins); Evanston; Niobrara; North Big Horn; South Big Horn; South Lincoln; Weston; Sublette County Hospital (Pinedale, obtained CMS approval as CAH as of February 2026); Community Hospital (Torrington); Hot Springs County; Johnson County; Converse County; Sweetwater County; Platte Valley; Powell Hospital; Star Valley; Washakie Medical Center; West Park Hospital.

Wyoming Initiative Component: Deliverable Incentive Payments			
INITIATIVE 1.1: Critical Access Hospital - Basic			
Deliverable	Allowable use of funds	Verification of deliverable	Year 1 incentive payment (payment made to CAH [unless otherwise noted] after verification that deliverable was met)
Provide a staffed 24/7 Emergency Department.	Appropriate care availability	Site visits and life-safety surveys, review of Medicaid claims data (to include dual eligibles)	\$250,000
Provide time-sensitive stroke and trauma imaging capability.	Appropriate care availability (if not previously extant or sufficient)	Site visits and life-safety surveys, review of Medicaid claims data.	\$250,000
Do not provide a designated set of elective conditions on-site.	Appropriate care availability	Site surveys, review of administrative claims data, coordination with private insurers.	\$250,000
Partnering with closest quality PPS hospital(s) through hub-and-spoke arrangement.	Fostering collaboration	Review of MOUs, meeting materials and patient flow records.	\$250,000
PPS hospital partnership in hub-and-spoke	Fostering collaboration	Review of MOUs and patient flows.	\$250,000, paid to PPS
Own and operate the only ground ambulance service within 35 miles, as defined by Medicare.	Appropriate care availability	Office of EMS site visits and verification in the Wyoming Ambulance Trip Reporting System (WATRs)	\$750,000
Offer basic labor and delivery service for mothers, if births in the county exceed designated threshold per year.	Appropriate care availability	Vital Statistics Data on births by hospital.	\$750,000
Offer renal dialysis to local residents with ESRD, either on-site or through a home-based model.	Appropriate care availability	Review of Medicaid claims data (to include dual-eligibles), site visits.	\$250,000

Funds are paid upon accomplishment of the deliverables. There are no restrictions on what provider can use funds for, once received.

Wyoming Initiative Component: Start-up Funding		
INITIATIVE 1.2: EMS Regionalization		
Startup cost (prioritized)	Use of funds	Budgeted amount (of total \$20 million pool)
Ambulance service acquisition and merger costs	Fostering collaboration	Up to \$400,000 per provider, one time (est. up to 20)
New ambulances	Appropriate care availability if not extant or insufficient.	Up to \$500,000 per provider (est. up to 20)
Interoperable dispatch equipment and integrated billing/revenue management systems	Fostering collaboration	Up to \$100,000 per provider (est. up to 20)

Wyoming Initiative Component: Eligible Applicants	
INITIATIVE 1.2: EMS Regionalization	
Provider Type	Provider Names
EMS Agency (Ground) (44 Total)	Albin Rescue; American Medical Response - Laramie County; Banner Health Paramedic Services - Platte County; Bar Nunn Fire Department; BHFD #4 Ambulance; Campbell County Health EMS; Carbon County EMS; Castle Rock Hospital District-Emergency Services; Cody Regional Health EMS; Crook County Medical Services District EMS; Eden Farson Fire District; Evansville Emergency Services; Frontier Ambulance; Glendo Volunteer Ambulance Service; Hawk Springs-FD; Hulett EMS; Jackson Hole Fire/EMS; Johnson County – Buffalo; Johnson County – Kaycee; LaGrange Fire Rescue; Laramie Fire Department; Lingle Fire Department; Little Snake River; Lusk EMS; Memorial Hospital of Converse County; Mills Fire Department; Moorcroft Ambulance; North Big Horn Hospital Ambulance; Pine Bluffs EMS; Powell Hospital Ambulance Service; Rawlins Fire Department; Salt Creek Emergency Services; Sheridan Fire-Rescue; South Central WY Emergency Medical Services; South Lincoln EMS; Star Valley Health EMS; Sublette County EMS; Ten Sleep Ambulance Service; Torrington Emergency Medical Services; Town of Pine Haven – EMS; Uinta County Fire/EMS; Upton-FD; Wamsutter EMS; Wyoming Medical Center.

Wyoming Initiative Component: Deliverable Incentive Payments			
INITIATIVE 1.2: EMS Regionalization			
Deliverable	Allowable use of funds	Verification of deliverable	Year 1 incentive payment (payment made to lead EMS entity [e.g. fire dept., CAH, county gov't] after verification that deliverable was met)
Provide 24/7 regionalized EMS capability (defined as the only ground ambulance service within 35 miles, per Medicare CAH rules)	Fostering collaboration	Site visits and WATRS ambulance registration data.	\$500,000
Maintain EMS response time (chute + drive time) below specified benchmark (e.g. ,< 30 minutes) in all frontier service areas.	Appropriate care availability	WATRS data on response times	\$500,000
Provide Community EMS services to non-Medicaid eligibles	Provider payments	WATRS CEMS reports	\$200,000

Funds are paid for accomplishment of the requirements and deliverables each year, with no other specific restrictions.

Wyoming RHT- Category F Contractor Breakdown			
Wyoming Budget Detail per Initiative Component, and Cost Category			
INITIATIVE 2: Rural Workforce Supply			
Initiative Component	Cost Type, Cost Category	FY 2026	% of Total RHT
2.1 Workforce education individual support	Direct, Contractor	\$35,424,820	15.47%
2.2 Physician Post-Graduate Education slots	Direct, Subrecipient	\$17,712,410	7.73%
2.3 Workforce education startup grants	Direct, Subrecipient	\$3,193,440	2.36%
2.4 RHT Policy Initiative- Licensure Compacts Physician Assistants	Direct, Subrecipient	\$100,000	0.02%
2.5 RHT Policy Initiative- Scope of Practice (Pharmacists)	Direct, Subrecipient	\$100,000	0.02%
2.6 RHT Policy Initiative- Scope of Practice (Dental Hygienist)	Direct, Subrecipient	\$100,000	0.02%
Initiative 2: Rural Workforce Supply	Direct, Subrecipient/Contractual	\$56,630,670	25.62%

Wyoming Initiative & Sub-Component Descriptions and Pertinent Information				
INITIATIVE 2: Rural Workforce Supply				
Initiative Component	Description of initiative	Procurement Expected	Allowable Bidder Pool	Expected Uses
2.1 Workforce education individual support	Educational support and stipends for physicians, registered nurses, mental health practitioners, certified nursing assistants, emergency medical technicians and paramedics. Includes direct tuition reductions, individual education awards, stipends, and other non-physician training. 5-year service requirement.	Competitive RFA	Physicians, RNs, mental health practitioners, CNAs, EMTs and Paramedics.	Other
2.2 Physician Post-Graduate Education slots	Physician post-graduate medical education slots added to residency and fellowship programs. 5-year service requirement in underserved areas.	Competitive RFA	Physicians in post-graduate medical education/residencies	Other
2.3 Workforce education startup grants	Medical career pipeline program for Wyoming high school students to promote medical careers and CNA and LPN certifications. Faculty recruitment to scale up slots for programs e.g. at University of Wyoming and WY Community Colleges, but may include out of state RFP applicants.	Competitive RFA	U.Wyo, Community Colleges, or other organizations offering education/training programs	Other
2.4 Policy Initiative- Licensure Compacts PAs	Wyoming currently has all but one licensure compact as described in the NOFO. These costs are associated with implementation of Physician's Assistant licensure compact.	Memorandum of Understanding (MOU) with Board of Medicine	Wyoming Board of Medicine	Supplies, Other
2.5 Policy Initiative- Scope of Practice (SoP) PharmD	Wyoming currently meets the expanded scopes for all but two (2) professions as described in the NOFO. These costs are associated with implementation of Pharmacist scope of practice changes.	MOU with Board of Pharmacy	Wyoming Board of Pharmacy	Supplies, Other
2.6 Policy Initiative- SoP Dental Hygien.	Wyoming currently meets the expanded scopes for all but two (2) professions as described in the NOFO. These costs are associated with implementation of Dental Hygienist scope of practice changes.	MOU with WY Board of Dental Examiners	Wyoming Board of Dental Examiners	Supplies, Other

Wyoming RHT- Category F Contractor Breakdown			
Wyoming Budget Detail per Initiative Component, and Cost Category by Federal Fiscal Year			
INITIATIVE 3: Health Technology Transformation			
Initiative Component	Cost Type, Cost Category	FY 2026	% of Total RHT
3.1 Technology adoption challenge	Direct, Contract	\$6,458,917	4.47%
3.2 Statewide tele-specialist platform	Direct, Contract	\$3,493,440	2.42%
Admin- Independent IT security- statewide tele-specialist platform	Direct, Contract	\$10,000	2.44%
3.3 Non-emergency transportation coordination platform	Direct, Contract	\$828,808	0.57%
Admin- Independent IT security- non-emergency transport coordination platform	Direct, Contract	\$10,000	0.00%
3.4 Centralized billing capability	Direct, Contract	\$2,075,168	1.44%
Admin- Independent IT security-centralized billing IT	Direct, Contract	\$10,000	0.00%
Initiative 3: Health Technology Transformation	Direct, Contract Costs	\$12,886,333	11.35%

Wyoming Initiative Component Descriptions and Pertinent Information				
INITIATIVE 3: Health Technology Transformation				
Initiative Component	Description of initiative	Procurement Expected	Allowable Bidder Pool	Expected Uses
3.1 Technology adoption challenge	Competitive grant process for technology procurement that promises to improve care delivery closer to home, build interoperability across providers, and cut wasteful administrative activity. Only groups of 2+ entities may apply, with preference for larger cooperative groups.	Competitive RFA	Wyoming health provider organizations.	Equipment (IT system), Contractual (IT)
3.2 Statewide tele-specialist platform	Competitive grant process promoting centralized tele-specialty, with focus on procuring psychiatry and other behavioral health services, to include a crisis stabilization telehealth capacity for emergencies. Target providers include for example: EDs, LEOs and jails, PRTFs, etc. , Budget includes any necessary independent security testing.	Competitive RFP	Any bidder able to deliver software and services as described in RFP.	Personnel, Fringe, Supplies, Equipment, Other
Admin-Contractual	Administration for contractual services for independent IT security architecture of new statewide tele-specialist platform.	Existing IT consulting contracts	Existing Medicaid IT contractors pursuant to RFP.	IT security contractors billed at contractual hourly rate
3.3 NEMT coordination platform	A statewide technology system for non-emergency transportation coordination, including any necessary independent security testing.	Competitive RFP	Any bidder able to deliver software and services as described in RFP.	Equipment (IT system), Contractual (IT)
Admin-Contractual	Security architecture for statewide technology system for non-emergency transportation coordination and routing.	Existing IT consulting contracts	Existing Medicaid IT contractors pursuant to RFP.	IT security contractors billed at contractual hourly rate
3.4 Centralized billing capability	A statewide technology system for non-emergency transportation billing, including any necessary independent security testing.	Competitive RFP	Any bidder able to deliver software and services as described in RFP.	Equipment (IT system), Contractual (IT)
Admin-Contractual	Security architecture for statewide technology system for non-emergency transportation billing.	Existing IT consulting contracts	Existing Medicaid IT contractors pursuant to RFP.	IT security contractors billed at contractual hourly rate

Wyoming RHT- Category F Contractor Breakdown			
Wyoming Budget Detail per Initiative Component, and Cost Category			
INITIATIVE 4: Make Wyoming Healthy Again			
Initiative Component	Cost Type, Cost Category	FY 2026	% of Total RHT
4.1 Integrated primary care expansion	Direct, Subrecipient	\$15,613,217	10.81%
4.2 Clinically-integrated care coordination	Direct, Subrecipient	\$2,989,078	2.38%
4.3 Exercise and healthy diet promotion - 4.3a programming, exercise/healthy diet promotion spaces and rooms	Direct, Subrecipient	\$3,046,562	2.53%
4.3 Exercise and healthy diet promotion-4.3b Presidential Fitness Test -Startup Cost of Rewards and Recognition Items	Direct, Subrecipient	\$200,000	0.04%
4.3 Exercise and healthy diet promotion- 4.3c SNAP Waiver startup and implementation costs	Direct, Subrecipient	\$684,525	0.21%
4.3 Exercise and healthy diet promotion-4.3d Policy Score Factor B.4- Nutrition CME Policy Change	Direct, Subrecipient	\$250,000	0.12%
Initiative 4: Make Wyoming Healthy Again	Direct, Subrecipient	\$22,783,382	16.09%

Wyoming Initiative & Sub-Component Descriptions and Pertinent Information				
INITIATIVE 4: Make Wyoming Healthy Again				
Initiative Component/ Sub-Component	Description of initiative	Procurement Expected	Allowable Bidder Pool	Expected Use of Funds
4.1 Integrated primary care expansion	Costs to competitively fund FQHC and like entities' costs in converting existing clinical spaces and practices to an integrated primary care model to expand the number and geographical reach FQHCs, with a focus on improving the integration of primary care with behavioral health, obstetric and gynecological care, as well as dental and preventive health services.	Comp. RFA	Wyoming FQHCs, Tribally run 638 clinics, other qualified organizations	Supplies, Equipment, Other
4.2 Clinically-integrated care coordination	Costs to competitively fund within each WY county, applicants e.g. hospitals, providers, and other rural health facilities, to improve clinical coordination of care for dual Medicare/Medicaid eligible at high risk of chronic disease.	Comp. RFA	Proposal should designate lead organization for each Wyoming county or region and include a broad coalition of providers.	Contracts, Supplies, Equipment
4.3a Exercise and healthy diet promotion	Costs for funding for exercise and diet promotion, small programming grants, and Centsible Nutrition. Equipment and supply projects examples: renovation of exercise rooms, wellness facilities at Senior Centers and Community Centers, refresh of walking areas (indoors or outdoors), or exercise areas at community organizations serving children.	Comp. RFA	Wyoming governments, non-profits, or commercial entities.	Personnel, Fringe, Supplies, Equipment, Other
4.3b Exercise and healthy diet promotion	Presidential Fitness Test -Startup Cost of Rewards and Recognition Items statewide for five years. Then program will be budgeted per normal State of Wyoming processes.	MOU with WDE	Wyoming Department of Education (WDE) for distribution to school districts.	Supplies
4.3c Exercise and healthy diet promotion	SNAP Waiver startup and implementation costs.	MOU with WDFS	Wyoming Department of Family Services (WDFS)	Personnel, Fringe, Supplies, Consultant
4.3d Exercise and healthy diet promotion	Policy Score Factor B.4- Implementation costs to provide nutrition training, and changes at Board of Medicine to provide, track, and require Nutrition CME.	MOU with Board of Medicine	Wyoming Board of Medicine	Other (CME training)

Wyoming RHT - G. Construction

Not applicable to Wyoming's RHT proposal.

Wyoming RHT - H. Other

Other Type	# Items Per Yr	\$ Per Item	Other Description	FY 2026
Printing	91,250	\$0.04	Printing for contract letters, public awareness mailings to target groups	\$3,650
Postage	25,000	\$0.78	Postage for contract letters, member correspondence (Clinically Integrated Care Coordination), Public awareness mailings to target groups	\$19,500
Postage (Flat Rate Priority)	500	\$11.90	Contracts and other flat envelopes, contract documents and SLA type enforcement	5,950.00
Phone Line	15	\$700.00	Landline (VOIP) office phones for 15 RHT employees	\$10,500
Total Other				\$39,600

Wyoming RHT - I. Direct Costs Summary

Direct	FY 2026	Direct Costs (%)
Direct	\$205,004,742	100.00%
Total Expenditures	\$205,004,742	100.0%

Wyoming RHT - J. Indirect Costs Summary

\$0 indirect costs are allocated to this proposal, equalling a 0.00% indirect cost rate. This decision was made intentionally to increase expenditures on direct initiatives and limit the administrative percentage in this proposal.

Wyoming RHT Proposal Compared to Applicable NOFO Limitations			
Type of Limit	NOFO Limit %	Wyoming Proposal	Initiatives Related to this NOFO Limit
Indirect Costs (15% without Federally approved cost allocation plan, or % approved in Agency cost allocation plan)	WDH PACAP (22%).	0%, \$0	Not Applicable. Wyoming intentionally did not allocate indirect costs to maximize transformation funds going directly to Wyoming communities.
State Administration Costs	10%	<1%	Administration
Personnel Wage Limit	\$250,600	>\$125,000	Administration
Technology EHR Replacement -HITECH Certified	5%	<5%	3.1 Technology adoption challenge. EHR replacement of a HITECH certified system may be an allowable use with justification on why system not meeting business needs, and how related to Wyoming's RHT goals. Wyoming is only aware of one hospital meeting this definition that may need to replace an EHR.
Technology EHR Replacement -Not HITECH Certified	No limitation.	Not applicable.	3.1 Technology adoption challenge (initiative component at 4.56% of total funding, not all spending will be on EHR upgrade or replacement items)
Renovations or alterations to buildings and infrastructure,if such expenditure will "ensure long-term overhead and upkeep costs are commensurate with patient volume, subject to restrictions."	20%	20% expected.	Proposals in these initiative areas may include renovations or alterations: 1.1 Critical Access Hospital - Basic 1.2 EMS regionalization 4.1 Integrated primary care expansion 4.3a Exercise and healthy diet promotion
Rural Tech Catalyst Fund Initiative	Lesser of 10% or \$20M	0%, \$0	Not applicable
Cumulative administrative costs, including those incurred by both the awardee and any subrecipient.	10%	<10%	All initiatives. Most planned procurements do not allow subrecipient administrative costs to be charged to RHT. No procurements allow allocation of indirect costs.
Provider Payments (Use of Funds Category B)	15%	2%	Provider payments in initiative 1.2 EMS Regionalization related to community paramedicine